

## Doctor of Medicine

Back

Time left 2:59:18

### Question 5

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 [Flag question](#)

A 17-year-old boy presented with high grade fever, nausea, vomiting of ingested matter and flank pain of three days duration. His temperature is 38.5°C, and pulse rate is 100/min. However, other physical examination findings are not remarkable. Laboratory result is notable for leukocytosis and pyuria. What is the most likely diagnosis of the boy?

- ☐ a. Acute cystitis
- ☐ b. Nephrolithiasis
- ☐ c. Acute prostatitis
- ☐ d. Acute pylenephritis

Previous page

Next page

Quiz navigation

[Back](#)

Time left 2:58:20

**Question 13**

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[Flag question](#)

A 24-year-old male patient came to an emergency OPD after he was injured at his back by an unknown offender three hours back. He had minimal bleeding from the stab site. On physical examination, he had 2 x 3cm deep laceration to his back, and the fascia and peritoneum were intact. What is the most appropriate management for the patient?

- ☐ a. Debridement, washing and primary closure
- ☐ b. Admit him, IV antibiotics and primary closure
- ☐ c. Wound care and delayed primary closure
- ☐ d. Wound care daily and let it close by itself

[Previous page](#)[Next page](#)**Quiz navigation**

|    |    |    |    |    |    |    |    |    |
|----|----|----|----|----|----|----|----|----|
| 1  | 2  | 3  | 4  | 5  | 6  | 7  | 8  | 9  |
| 10 | 11 | 12 | 13 | 14 | 15 | 16 | 17 | 18 |

## Doctor of Medicine

[Back](#)

Time left 2:58:31

### Question 11

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[Flag question](#)

A 22-year-old male patient presented with progressive mid-abdominal discomfort followed by anorexia, nausea and vomiting of 24 hours duration. The pain later shifted to the right iliac fossa. He had right lower quadrant abdominal tenderness and WBC count of  $12000/\text{mm}^3$  with left shift (NR;  $3.54 - 9.06 \times 10^3/\text{mm}^3$ ). What is the ultrasound finding that supports the most likely diagnosis?

- ☐ a. Presence of fluid in general peritoneum
- ☐ b. Length of appendix greater than 15mm
- ☐ c. Presence of air in general peritoneum
- ☐ d. Diameter of appendix greater 6mm

[Previous page](#)[Next page](#)

Quiz navigation

[Back](#)

Time left 2:56:26

**Question 27**

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[Flag question](#)

A 28-year-old male patient came to a health center after noticing skin discoloration on his back for unknown duration. Five hypo-pigmented skin lesions with loss of sensation on the lesion and enlarged ulnar nerves were found during examination. What is the most likely diagnosis?

- ☐ a. Leprosy
- ☐ b. Ulnar nerve neuropathy
- ☐ c. Tinea corporis
- ☐ d. Pityriasis alba

[Previous page](#)[Next page](#)**Quiz navigation**

|    |    |    |    |    |    |    |    |    |
|----|----|----|----|----|----|----|----|----|
| 1  | 2  | 3  | 4  | 5  | 6  | 7  | 8  | 9  |
| 10 | 11 | 12 | 13 | 14 | 15 | 16 | 17 | 18 |
| 19 | 20 | 21 | 22 | 23 | 24 | 25 | 26 | 27 |



Time left 2:56:19

**Question 28**

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 [Flag question](#)

A 29-year-old G-II, P-I mother, who is a known Graves's disease patient on treatment, presented to the regular OPD at her 36th weeks of gestation with complaints of palpitation, nervousness, sweating, and diarrhea of one day duration. On physical examination, she is anxious and confused, her blood pressure is 150/90 mmHg, pulse rate is 130/min, respirations are 28/min, temperature is 38.3°C and there is systolic ejection murmur at the apex. CBC results were; hematocrit level of 39percent, WBC count of  $18 \times 10^3/\text{mm}^3$ , and platelet count of  $260 \times 10^3/\text{mm}^3$ . What is the most likely diagnosis of this patient?

- ☐ a. Myxedema coma
- ☐ b. Thyroid storm
- ☐ c. Eclampsia
- ☐ d. Adrenal crisis

Previous page

Next page



Time left 2:54:46

**Question 41**

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A 35-year-old female patient came to a hospital with anterior neck swelling of five years duration. Since the last three month, she has started to experience palpitation and has a history of hot intolerance with no history of bone pain, cough or yellowish discoloration of the eye. On examination, her pulse rate is 110/min, blood pressure is 120/70mmHg, and there is lid lag and lid retraction. On thyroid examination, there is diffusely enlarged thyroid measuring of 4\*3cm;the lower border is reachable. She is investigated with thyroid function test and the results show that TSH is 0.3micro unit/mL(Normal; 0.5–5 micro unit/mL), Total T4 is 180 nmol/L(Normal; 55–150 nmol/L), Total T3 is 5 nmol/L(Normal; 1.5–3.5 nmol/L). What is the most likely diagnosis of the patient?

- ☐ a. Toxic multinodular goiter
- ☐ b. Toxic Adenoma
- ☐ c. Graves' disease
- ☐ d. Follicular neoplasm



## Doctor of Medicine

[Back](#)

Time left 2:55:10

### Question 38

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[Flag question](#)

A 34-year-old male patient sustained stick injury to his arm. He had pain and is unable to use his extremity. On evaluation, his vital signs are in the normal range. There is visible deformity and crepitus over the left arm. Distal pulses are intact and there is wrist drop. What is the most likely explanation for this case?

- ☐ a. Humeral shaft fracture plus radial nerve injury
- ☐ b. Humeral shaft fracture plus radial and ulnar nerve injury
- ☐ c. Humeral shaft fracture plus median nerve injury
- ☐ d. Humeral shaft fracture plus ulnar nerve injury



Time left 2:54:35

**Question 42**

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A 35-year-old female patient presents with colicky right upper quadrant abdominal pain of three months duration. Her pain worsens when she eats fatty meal. Since four days ago, she has persistent right upper quadrant pain, vomiting of ingested matter and fever. On physical examination, her blood pressure is 100/60mmHg, pulse rate is 112/min, temperature is 38.7 degree centigrade and she has tenderness during inspiration on right upper quadrant abdomen. On investigation WBC count was 12,000/mm<sup>3</sup>, and abdominal ultrasound shows gall bladder wall of 3.5mm thick and hyperechoic lesion with posterior shadow within it. What is the best initial management for this patient?

- ☐ a. Maintenance fluid, keep NPO and start sips when she tolerate
- ☐ b. Maintenance fluid, IV antibiotics, and urgent cholecystectomy
- ☐ c. Maintenance fluid, PO antibiotics and analgesics for seven days
- ☐ d. Maintenance fluid, IV antibiotics, analgesics and bowel rest

## Doctor of Medicine

[Back](#)

Time left 2:53:33

### Question 49

Not yet answered

Marked out of 1.00

 [Flag question](#)

A 36-year-old woman who was operated for longstanding goiter presented to a surgical referral clinic with paresthesias of the distal extremities, numbness, painful muscle spasms, and seizures in the 3rd postoperative day. What is the most likely electrolyte abnormality in this case?

- ☐ a. Hypokalemia
- ☐ b. Hypocalcemia
- ☐ c. Hyponatremia
- ☐ d. Hypomagnesemia

[Previous page](#)[Next page](#)

### Quiz navigation

1

2

3

4

5

6

7

8

9



[Back](#)

Time left 2:52:55

**Question 53**

Not yet answered

Marked out of 1.00

 [Flag question](#)

A 40-year-old female patient came to an OPD with the complaint of amenorrhea of three months duration. She also had frequent abortion for which she visited many private clinics and did not get cure. She also complained hot intolerance and significant weight loss. Her pulse rate is 120/min. On examination, she has lid lag and exophthalmia, diffused anterior neck swelling which moves with respiration, and thickened skin over the dorsum of the foot and pre-tibial region. What is the most likely explanation for these findings?

- ☐ a. Hashimoto's thyroiditis
- ☐ b. Secondary thyrotoxicosis
- ☐ c. Subacute thyroiditis
- ☐ d. Graves disease

[Previous page](#)[Next page](#)**Quiz navigation**

[Back](#)

Time left 2:52:15

**Question 57**

Not yet answered

Marked out of 1.00

[Flag question](#)

A 48-year-old female patient was admitted to a medical ICU with the diagnosis of NYHA Class-IV Stage-C CHF secondary to chronic rheumatic valvular heart disease. The physicians and nurses in the ICU were unable to secure IV line. Where is the most appropriate site for IV access in the patient?

- ☐ a. Saphenous vein cut down
- ☐ b. Intraosseous route
- ☐ c. Scalp vein
- ☐ d. Jugular vein cut down

[Previous page](#)[Next page](#)**Quiz navigation**

|    |    |    |    |    |    |    |    |    |
|----|----|----|----|----|----|----|----|----|
| 1  | 2  | 3  | 4  | 5  | 6  | 7  | 8  | 9  |
| 10 | 11 | 12 | 13 | 14 | 15 | 16 | 17 | 18 |
| 19 | 20 | 21 | 22 | 23 | 24 | 25 | 26 | 27 |

[Back](#)

Time left 2:51:40

**Question 61**

Not yet answered

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[Flag question](#)

A 54-year-old man came to a hospital with a complaint of vomiting of coffee ground matter of five hours duration. He has long standing epigastric pain, easy fatigability and dark tarry stool. At presentation, his blood pressure is 85/60mmHg and pulse rate is 120/min. His abdominal examination did not reveal remarkable finding. His laboratory findings show WBC count of  $4.3 \times 10^3/\text{mm}^3$ , platelet count of  $178 \times 10^3$  and hemoglobin level of 7g/dl. What is the most appropriate initial management for the patient?

- ☐ a. Resuscitate and prepare for emergency laparotomy
- ☐ b. Secure IV line and send for upper GI endoscopy immediately
- ☐ c. Resuscitate with double IV line and prepare cross matched blood
- ☐ d. Put him on maintenance fluid and do NG tube lavage

[Previous page](#)[Next page](#)



Time left 2:51:16

**Question 64**

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[Flag question](#)

A 55-year-old male patient presented to an emergency OPD with abdominal pain, distension, vomiting and failure to pass feces and flatus of three days duration. He had swelling on the left inguinal region for the last five years, which failed to reduce since four days. His physical examination results show a blood pressure of 100/60 mmHg, pulse rate of 112/min, temperature of 38.1 degree centigrade, abdomen is distended, and hypertympanic with decreased bowel sound, and there is a 12 cm x 15 cm non-reducible tender mass on the left inguinal region. What is the most likely diagnosis?

- ☐ a. Strangulated inguinal hernia
- ☐ b. Obstructed inguinal hernia
- ☐ c. Sliding inguinal hernia
- ☐ d. Incarcerated inguinal hernia

[Previous page](#)[Next page](#)**Quiz navigation**

[Back](#)

Time left 2:50:51

**Question 67**

Not yet answered

Marked out of 1.00

[Flag question](#)

A 58-year-old man with abdominal distension and pain of two weeks duration visited a hospital. He also has blood mixed stool, tenesmus and weight loss. He was treated for intestinal parasitosis multiple times. On physical examination, his vital signs were within the normal range, abdomen was distended and hyper-tympanic, and bowel sounds were hyperactive. His hematocrite level was 32percent. Plain abdominal X-ray showed distended large bowel and barium enema showed apple core appearance. What is the most likely diagnosis of the patient?

- ☐ a. Colorectal Cancer
- ☐ b. Transverse Colon Volvulus
- ☐ c. Sigmoid Volvulus
- ☐ d. Cecal Volvulus

[Previous page](#)[Next page](#)**Quiz navigation**



## Doctor of Medicine

[Back](#)

Time left 2:50:33

### Question 69

Not yet answered

Marked out of 1.00

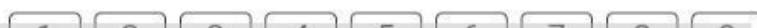
[Flag question](#)

A 60-year-old male patient needs to undergo urgent surgical intervention for small bowel obstruction. He was on aspirin for the last three years. What is the best component of blood that should be prepared in this patient care?

- ☐ a. Packed RBC
- ☐ b. Cryoprecipitate
- ☐ c. Whole blood
- ☐ d. Platelet

[Previous page](#)[Next page](#)

### Quiz navigation





## Doctor of Medicine

[Back](#)

Time left 2:49:57

### Question 73

Not yet answered

Marked out of 1.00

[Flag question](#)

A 75-year-old female patient came to a surgical OPD with complaints of difficulty of swallowing for three months. She also had hoarseness of voice. On evaluation, she was chronically sick looking. She had left supraclavicular LAP. What is the most appropriate initial diagnosis for this patient?

- ☐ a. Achalasia
- ☐ b. Lymphoma
- ☐ c. Esophageal cancer
- ☐ d. Tuberculous LAP

[Previous page](#)[Next page](#)

## Doctor of Medicine

Back

Time left 2:48:29

### Question 83

Not yet answered

Marked out of 1.00

 [Flag question](#)

A patient who had been diagnosed with pulmonary tuberculosis came to an emergency room with a complaint of difficulty of breathing and chest pain. He is tachypenic and has signs of fluid collection on the right side which was tapped and found to be grossly purulent. His physician decided to insert chest tube. Where is the safest site of chest tube insertion on this patient?

- ☐ a. Fifth Intercostal space anterior to the mid-axillary line
- ☐ b. Fifth Intercostal space anterior to the anterior -axillary line
- ☐ c. Second Intercostal space below and lateral to the pectoralis major muscle
- ☐ d. Seventh Intercostal space below the level of the nipple

Previous page

Next page

## Doctor of Medicine

Back

Time left 2:45:53

### Question 99

Not yet answered

Marked out of 1.00

 [Flag question](#)

An 18-year-old female patient came to a surgical OPD with a complaint of breast lump of three weeks duration. She has no pain and family history of similar illness. On evaluation, her vital signs are in the normal range and there is a freely mobile non-tender firm 2 x 2cm mass at right outer upper quadrant. What is the most appropriate next step in the patient's care?

- ☐ a. Send her for ultrasound and FNAC
- ☐ b. Reassure her and appoint after three months for evaluation
- ☐ c. Tell her it is fibroadenoma and send her home with analgesia
- ☐ d. Send her for mammography



## Doctor of Medicine

[Back](#)

Time left 2:45:44

### Question 100

Not yet answered

Marked out of 1.00

[Flag question](#)

An old diabetic patient is going to be operated for large bowel obstruction secondary to sigmoid volvulus. He was on oral glycemc agents for the last five years. No other medications. His vital sign is in the normal range. What is the best preoperative preparation for this patient?

- ☐ a. Making his blood sugar above 180mg/dl because of Intraoperative hypoglycemia
- ☐ b. Changing oral glycemc agents to regular insulin
- ☐ c. His blood sugar should be as low as possible due to risk of stress induced hyperglycemia
- ☐ d. He has to take morning dose of oral hypoglycemic agents with regular insuline

[Previous page](#)[Finish attempt ...](#)